Humana. **Claims Payment Policy**

Subject: Multiple Procedure Payment Reduction (MPPR)

Application: Medicare Advantage Products
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Overview

This policy outlines various multiple procedure payment reductions (MPPR). Practitioner reimbursement for a procedure code generally includes reimbursement for the individual costs, also known as inputs, typically associated with that service. For example, the Centers for Medicare & Medicaid Services (CMS) uses a formula that accounts for the inputs associated with a procedure code to determine Medicare Physician Fee Schedule (MPFS) reimbursement rates for that code. When the *same provider* renders multiple services, or multiple units of the same service, to the same patient on the same day, the total calculation for the procedure code(s) could account more than once for an input that was only rendered once. Examples of inputs that might be contemplated in reimbursement for certain procedure codes, that would not typically be duplicated in the same encounter include: greeting and gowning the patient; preparing the room, equipment and supplies; providing education and obtaining consent. An MPPR adjusts reimbursement to offset duplication of reimbursement for inputs that were only rendered once.

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In addition to the policy, claims payments are subject to other plan requirements for the processing and payment of claims, including, but not limited to, requirements of medical necessity and reasonableness and applicable referral or authorization requirements.

Diagnostic Cardiovascular Services

For diagnostic cardiovascular services, the portion of an in-network practitioner's contracted rate that is attributable to the *technical component (TC)* may be subject to MPPR. For diagnostic cardiovascular services rendered by an out-of-network practitioner, the portion of the *base maximum amount payable under the member's plan* that is attributable to the *TC* may be subject to MPPR.

For reimbursement of only a *TC*, all of the contracted rate or *base maximum amount payable under the member's plan* is attributable to the *TC*. For reimbursement of both a *TC* and a *professional component (PC)*, the portion of the contracted rate or *base maximum amount payable under the member's plan* that is attributable to the *TC* is that rate or amount multiplied by the ratio of the code's MPFS local *TC* amount to the code's MPFS local global amount. For example:

Procedure code	Contracted rate	MPFS local *TC* amount	MPFS local global amount	Ratio of: $\frac{\text{MPFS local } TC \text{ amount}}{\text{MPFS local global amount}}$	Portion attributable to *TC*
ccccc	$10	$16	$20	$\frac{16}{20}$ = 80%	$10 x 80% = $8

Humana applies an MPPR to same-day diagnostic cardiovascular *TC* services rendered by the *same provider*.

To determine whether the MPPR applies to a specific allowed charge or to one or more units of a charge with multiple allowed units:

First, of the services for which Humana is reimbursing for a *TC*, Humana identifies the same-day services with a *MPFS Relative Value File* multiple procedure indicator 6.

Procedure code	Units	Portion attributable to TC
aaaaa	1	$12 per unit
bbbbb	2	$20 per unit
ccccc	1	$ 8 per unit

Second, Humana ranks all identified services from highest dollar amount to lowest dollar amount. We rank by the portion of the charge's per-unit practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *TC*, as described above.

Procedure code	Units	Portion attributable to *TC*
bbbbb	2	$20 per unit
aaaaa	1	$12 per unit
ccccc	1	$ 8 per unit

Finally, Humana eliminates from review the unit of service that is ranked highest; or, if multiple units of service are ranked highest, Humana eliminates one of those units. Humana applies MPPR to all other allowed units of service.

When Humana determines to apply diagnostic cardiovascular services *TC* MPPR to a unit, a 25 percent reduction is applied to the reimbursement for the portion of the unit's practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *TC*.

Procedure code	Units	Portion attributable to *TC*	Reduction percentage	Amount allowed
bbbbb	1	$20 per unit	No reduction	$20
bbbbb	1	$20 per unit	25%	$15
aaaaa	1	$12 per unit	25%	$9
ccccc	1	$ 8 per unit	25%	$6

Note: In certain circumstances, additional payment rules may apply.

Diagnostic Imaging Services

Diagnostic Imaging Services Technical Component

For diagnostic imaging services, the portion of an in-network practitioner's contracted rate that is attributable to the *TC* may be subject to MPPR. For diagnostic imaging services rendered by an out-of-network practitioner, the portion of the *base maximum amount payable under the member's plan* that is attributable to the *TC* may be subject to MPPR.

For reimbursement of only a *TC*, all of the contracted rate or *base maximum amount payable under the member's plan* is attributable to the *TC*. For reimbursement of both a *TC* and a *PC*, the portion of the contracted rate or *base maximum amount payable under the member's plan* that is attributable to the *TC* is that rate or amount multiplied by the ratio of the code's MPFS local *TC* amount to the code's MPFS local global amount.

Humana applies an MPPR to same-day diagnostic imaging *TC* services rendered by the *same provider*.

To determine whether the MPPR applies to a specific allowed charge or to one or more units of a charge with multiple allowed units:

First, of the services for which Humana is reimbursing for a *TC*, Humana identifies the same-day services with a *MPFS Relative Value File* multiple procedure indicator 4.

Second, Humana ranks all identified services from highest dollar amount to lowest dollar amount. We rank by the portion of the charge's per-unit practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *TC*, as described above.

Finally, Humana eliminates from review the unit of service that is ranked highest; or, if multiple units of service are ranked highest, Humana eliminates one of those units. Humana applies MPPR to all other allowed units of service.

When Humana determines to apply diagnostic imaging services *TC* MPPR to a unit, a 50 percent reduction is applied to the reimbursement for the portion of the unit's practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *TC*.

Note: In certain circumstances, additional payment rules may apply.

Diagnostic Imaging Professional Component

For diagnostic imaging services, the portion of an in-network practitioner's contracted rate that is attributable to the *PC* may be subject to MPPR. For diagnostic imaging services rendered by an out-of-network practitioner, the portion of the *base maximum amount payable under the member's plan* that is attributable to the *PC* may be subject to MPPR.

For reimbursement of only a *PC*, all of the contracted rate or *base maximum amount payable under the member's plan* is attributable to the *PC*. For reimbursement of both a *PC* and a *TC*, the portion of the contracted rate or *base maximum amount payable under the member's plan* that is attributable to the *PC* is that rate or amount multiplied by the ratio of the code's MPFS local *PC* amount to the code's MPFS local global amount.

Humana applies an MPPR to same-day diagnostic imaging *PC* services rendered by the *same provider*.

To determine whether the MPPR applies to a specific allowed charge or to one or more units of a charge with multiple allowed units:

First, of the services for which Humana is reimbursing for a *PC*, Humana identifies the same-day services with a *MPFS Relative Value File* multiple procedure indicator 4.

Second, Humana ranks all identified services from highest dollar amount to lowest dollar amount. We rank by the portion of the charge's per-unit practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *PC*, as described above.

Finally, Humana eliminates from review the unit of service that is ranked highest; or, if multiple units of service are ranked highest, Humana eliminates one of those units. Humana applies MPPR to all other allowed units of service.

When Humana determines to apply diagnostic imaging services *PC* MPPR to a unit:

- For dates of service on or after January 1, 2017, a 5 percent reduction is applied to the reimbursement for the portion of the unit's practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *PC*.
- For dates of service before January 1, 2017, a 25 percent reduction is applied to the reimbursement for the portion of the unit's practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *PC*.

Note: In certain circumstances, additional payment rules may apply.

Diagnostic Imaging Technical and Professional Component
Any charge could be subject to a *TC* MPPR, *PC* MPPR or both. When both *TC* MPPR and *PC* MPPR are applied, calculation of the *TC* MPPR is separate from calculation of the *PC* MPPR. The rankings of *TC* and *PC* are independent of each other. For example:

First, of the services for which Humana is reimbursing for a *TC* and *PC*, Humana identifies the same-day services with a *MPFS Relative Value File* multiple procedure indicator 4.

Procedure code	Units	Portion attributable to *TC*	Portion attributable to *PC*
aaaaa	1	$20 per unit	$40 per unit
bbbbb	1	$40 per unit	$20 per unit

Second, Humana ranks all identified services from highest dollar amount to lowest dollar amount. We rank by the portion of the charge's per-unit practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *TC* and *PC* separately, as described above.

Procedure code	Units	Portion attributable to *TC*
bbbbb	1	$40 per unit
aaaaa	1	$20 per unit

Procedure code	Units	Portion attributable to *PC*
aaaaa	1	$40 per unit
bbbbb	1	$20 per unit

Finally, Humana eliminates from review the unit of service that is ranked highest for *TC* and *PC* separately. Humana applies *TC* MPPR and *PC* MPPR separately to all other allowed units of service.

When Humana determines to apply diagnostic imaging services *TC* MPPR to a unit, a 50 percent reduction is applied to the reimbursement for the portion of the unit's practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *TC*.

When Humana determines to apply diagnostic imaging services *PC* MPPR to a unit, for dates of service on or after January 1, 2017, a 5 percent reduction is applied to the reimbursement for the portion of the unit's practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *PC*.

Procedure code	Units	*TC* ranking	*PC* ranking	Portion attributable to *TC*	Portion attributable to *PC*	Reduction percentage for *TC*	Reduction percentage for *PC*	Amount allowed for *TC*	Amount allowed for *PC*	Amount allowed
aaaaa	1	Second	First	$20 per unit	$40 per unit	50%	No reduction for *PC*	$10 per unit	$40 per unit	$50
bbbbb	1	First	Second	$40 per unit	$20 per unit	No reduction for *TC*	5%	$40 per unit	$19 per unit	$59

Diagnostic Ophthalmology Services

For diagnostic ophthalmology services, the portion of an in-network practitioner's contracted rate that is attributable to the *TC* may be subject to MPPR. For diagnostic ophthalmology services rendered by an out-of-network practitioner, the portion of the *base maximum amount payable under the member's plan* that is attributable to the *TC* may be subject to MPPR.

For reimbursement of only a *TC*, all of the contracted rate or *base maximum amount payable under the member's plan* is attributable to the *TC*. For reimbursement of both a *TC* and a *PC*, the portion of the contracted rate or *base maximum amount payable under the member's plan* that is attributable to the *TC* is that rate or amount multiplied by the ratio of the code's MPFS local *TC* amount to the code's MPFS local global amount. For example:

Procedure code	Contracted rate	MPFS local *TC* amount	MPFS local global amount	Ratio of: $\frac{\text{MPFS local } TC \text{ amount}}{\text{MPFS local global amount}}$	Portion attributable to *TC*
ccccc	$10	$16	$20	$\frac{16}{20}$ = 80%	$10 x 80% = $8

Humana applies an MPPR to same-day diagnostic ophthalmology *TC* services rendered by the *same provider*.

To determine whether the MPPR applies to a specific allowed charge or to one or more units of a charge with multiple allowed units:

First, of the services for which Humana is reimbursing for a *TC*, Humana identifies the same-day services with a *MPFS Relative Value File* multiple procedure indicator 7.

Procedure code	Units	Portion attributable to *TC*
aaaaa	1	$10 per unit
bbbbb	2	$20 per unit
ccccc	1	$ 5 per unit

Second, Humana ranks all identified services from highest dollar amount to lowest dollar amount. We rank by the portion of the charge's per-unit practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *TC*, as described above.

Procedure code	Units	Portion attributable to *TC*
bbbbb	2	$20 per unit
aaaaa	1	$10 per unit
ccccc	1	$ 5 per unit

Finally, Humana eliminates from review the unit of service that is ranked highest; or, if multiple units of service are ranked highest, Humana eliminates one of those units. Humana applies MPPR to all other allowed units of service.

When Humana determines to apply diagnostic ophthalmology services *TC* MPPR to a unit, a 20 percent reduction is applied to the reimbursement for the portion of the unit's practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *TC*.

Procedure code	Units	Portion attributable to *TC*	Reduction percentage	Amount allowed
bbbbb	1	$20 per unit	No reduction	$20
bbbbb	1	$20 per unit	20%	$16
aaaaa	1	$10 per unit	20%	$8
ccccc	1	$ 5 per unit	20%	$4

Note: In certain circumstances, additional payment rules may apply.

Therapy *Practice Expense (PE)*

For *always therapy* services, the portion of an in-network practitioner's contracted rate that is attributable to the *PE* may be subject to MPPR. For *always therapy* services rendered by an out-of-network practitioner, the portion of the *base maximum amount payable under the member's plan* that is attributable to the *PE* may be subject to MPPR.

The portion of the contracted rate or *base maximum amount payable under the member's plan* that is attributable to the *PE* is that rate or amount multiplied by the ratio of: the code's MPFS local *PE* amount to the code's MPFS local global amount.

The formula for a code's MPFS local *PE* amount is: (*PE RVU* x *PE GPCI*) x *CF*

For example, if, for a particular locale, the values for a date of service, for procedure code ccccc are:

- *PE RVU*: 0.25
- *PE GPCI*: 2.0
- *CF*: $32.00

Then, the code's MPFS local *PE* amount is: (0.25 x 2.0) x $32.00 = $16.00

Insert this value into the calculation described above for determining the portion attributable to *PE* as follows:

Procedure code	Contracted rate	MPFS local *PE* amount	MPFS local global amount	Ratio of: $\frac{\text{MPFS local } PE \text{ amount}}{\text{MPFS local gobal amount}}$	Portion attributable to *PE*
ccccc	$10	$16	$20	$\frac{16}{20}$ = 80%	$10 x 80% = $8

Humana applies an MPPR to same-day *always therapy* services rendered by the *same provider*.

To determine whether the MPPR applies to a specific allowed charge or to one or more units of a charge with multiple allowed units:

First, Humana identifies the same-day services with a *MPFS Relative Value File* multiple procedure indicator 5.

Procedure code	Units	Portion attributable to *PE*
aaaaa	1	$12 per unit
bbbbb	2	$20 per unit
ccccc	1	$ 8 per unit

Second, Humana ranks all identified services from highest dollar amount to lowest dollar amount. We rank by the portion of the charge's per-unit practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *PE*, as described above.

Procedure code	Units	Portion attributable to *PE*
bbbbb	2	$20 per unit
aaaaa	1	$12 per unit
ccccc	1	$ 8 per unit

Finally, Humana eliminates from review the unit of service that is ranked highest; or, if multiple units of service are ranked highest, Humana eliminates one of those units. Humana applies MPPR to all other allowed units of service.

When Humana determines to apply therapy *PE* MPPR to a unit, a 50 percent reduction is applied to the reimbursement for the portion of the unit's practitioner's contracted rate or *base maximum amount payable under the member's plan* that is attributable to *PE*.

Procedure code	Units	Portion attributable to *PE*	Reduction percentage	Amount allowed
bbbbb	1	$20 per unit	No reduction	$20
bbbbb	1	$20 per unit	50%	$10
aaaaa	1	$12 per unit	50%	$6
ccccc	1	$ 8 per unit	50%	$4

Note: In certain circumstances, additional payment rules may apply.

Multiple Endoscopic Procedures

For endoscopic services rendered by an in-network practitioner, the contracted rate may be subject to MPPR. For endoscopic services rendered by an out-of-network practitioner, the *base maximum amount payable under the member's plan* may be subject to MPPR.

Humana applies an MPPR to same-family endoscopic procedures, including multiple units of the same procedure code, rendered by the *same provider* on the same day.

To determine whether the MPPR applies to a specific allowed charge:

First, of the services for which Humana is reimbursing, Humana identifies, for a date of service, all *MPFS Relative Value File* multiple procedure indicator 3 endoscopy codes and identifies each service's base endoscopy code.

Procedure code	Base code
aaaaa	yyyyy
bbbbb	zzzzz
ccccc	yyyyy
ddddd	N/A
yyyyy	yyyyy

Second, Humana groups all services with the same endoscopy base code. Each group of services with the same base code is one endoscopic family.

Procedure code	Base code	Endoscopic family
aaaaa	yyyyy	Family 1
ccccc	yyyyy	Family 1
yyyyy	yyyyy[1]	Family 1
bbbbb	zzzzz	Family 2
ddddd	N/A	N/A

1. An endoscopy base code serves as its own base code.

Third, when there are multiple procedure codes for an endoscopic family and one of those is the family's base code, Humana denies separate reimbursement for that base code charge. Reimbursement for the base code is included in reimbursement for the highest ranked procedure code in that endoscopic family.

Procedure code	Base code	Endoscopic family
aaaaa	yyyyy	Family 1
ccccc	yyyyy	Family 1
~~yyyyy~~[1]	yyyyy	Family 1
bbbbb	zzzzz	Family 2
ddddd	N/A	N/A

1. The endoscopy base code is removed from the chart because it is not separately reimbursable.

Humana ranks all reimbursable services within each endoscopic family by the procedure's contracted rate or *base maximum amount payable under the member's plan*.

Procedure code	Base code	Endoscopic family	Procedure code base amount	Endoscopic family rank
ccccc	yyyyy	Family 1	$1000	Highest (Family 1)
aaaaa	yyyyy	Family 1	$850	Second (Family 1)
bbbbb	zzzzz	Family 2	$750	Highest (Family 2)
ddddd	N/A	N/A	$ 1000	N/A

Finally, within each endoscopic family, Humana reduces reimbursement for each procedure code, except that family's highest ranked procedure. The amount Humana uses to reduce reimbursement for a procedure code is the amount allowed for the procedure's base code.

Procedure code	Base code	Endoscopic family	Fee schedule amount	Endoscopic family rank	Amount allowed for base endoscopy procedure code	Amount allowed after multiple endoscopy reduction
ccccc	yyyyy	Family 1	$1000	Highest	$400	$1000
aaaaa	yyyyy	Family 1	$850	Second	$400	$450
bbbbb	zzzzz	Family 2	$750	Highest	$300	$750
ddddd	N/A	N/A	$ 1000	N/A	N/A	N/A

When a contracted rate does not include an amount allowed for a base code, Humana will apply the following formula based on amounts provided by CMS. These amounts are used to create a reduction amount. The reduction amount is then applied to the contracted rate.

Procedure code	Contracted rate	CMS fee schedule amount	Base code	CMS fee schedule amount for the base code	Ratio	Reduction amount	Final amount allowed
bbbbb	$1000	$850	yyyyy	$400	$400/$850=47.06%	$1000*47.06%=$470.60	$1000-$470.60=$529.40

Endoscopic procedures are also subject to multiple surgery reduction. As discussed in the Multiple Surgery section below, each endoscopic family is treated as a single surgery for determination of a multiple surgery reduction.

Procedure code	Base code	Endoscopic family	Amount allowed after multiple endoscopy reduction	Amount used for multiple surgery ranking	Ranking for multiple surgery	Multiple surgery reduction percentage	Final amount allowed[1]
ccccc	yyyyy	Family 1	$1000	$1450	Highest	N/A	$1000
aaaaa	yyyyy	Family 1	$450	$1450	Highest	N/A	$450
bbbbb	zzzzz	Family 2	$750	$750	Third	50%	$375
ddddd	N/A	N/A	N/A	$1000	Second	50%[1]	$500

1. As used in this example, the multiple surgery reduction percentage is based on the Medicare multiple surgery reduction. See Multiple Surgery section for more information.

Multiple Surgery

Humana applies a multiple surgery reduction when:

- The *same provider* performs more than one same-day *MPFS Relative Value File* multiple procedure indicator 2 service; or
- The *same provider* performs endoscopic procedures in more than one endoscopic family on the same day (see Multiple Endoscopic Procedures section above); or
- The *same provider* performs at least one *MPFS Relative Value File* multiple procedure indicator 2 service and at least one endoscopic procedure on the same day.

In unique circumstances, for the purposes of applying a multiple surgery reduction:

- Humana may not consider an *MPFS Relative Value File* multiple procedure indicator 2 service as a surgery.
- Humana may consider a service that does not have an *MPFS Relative Value File* multiple procedure indicator of 2 as a surgery.

Multiple surgery reduction applies to same-day services whether or not those services were submitted with *modifier 51*, *modifier 59* or *modifiers X{EPSU}.*

Humana plans apply a multiple surgery reduction as follows:

- The procedure with the highest contracted rate or *base maximum amount payable under the member's plan* is not reduced; and
- All other procedures are reduced by 50 percent.

Procedure code	Units	Base amount	Ranking	Medicare reduction percentage	Medicare amount allowed
aaaaa	1	$500	Highest	no reduction	$500
bbbbb	1	$300	Second	50%	$150
ccccc	1	$200	Third	50%	$100

Special Rules for Endoscopic Services
For an endoscopic service, multiple endoscopy reduction and multiple surgery reduction are determined separately. This will result in one of the following scenarios, depending on the number of services billed and the individual multiple procedure indicator code of each service**:**

- Multiple endoscopy reduction alone could apply to the service.
- Multiple surgery reduction alone could apply to the service.
- Both multiple endoscopy reduction and multiple surgery reduction could apply to the service.
- Neither multiple endoscopy reduction nor multiple surgery reduction could apply to the service.

If multiple endoscopy reduction applies, it is always determined before multiple surgery reduction.

After determining any multiple endoscopy reductions, all endoscopic services in the same endoscopic family are treated as a single service for the purpose of determining multiple surgery reduction, as follows:

- For multiple surgery reduction ranking purposes, endoscopic procedures in the same endoscopic family are treated as one service.
- An endoscopic family's multiple surgery reduction ranking establishes the multiple surgery reduction percentage, if any, for all services in that endoscopic family.

Note: Additional payment rules may apply in certain circumstances. For example, when a bilateral adjustment is applicable, it will be applied before a multiple surgery reduction.

Definitions of *Italicized* Terms

- ***Always Therapy:*** A service that is always considered a therapy, as noted in the *Medicare Physician Fee Schedule (MPFS) Relative Value File* by a multiple procedure indicator 5.
- ***Base maximum amount payable under the member's plan:*** The maximum amount of reimbursement for a unit of a covered expense, determined by the member's plan. Depending on the plan, this may be the maximum allowable fee, the appropriate Medicare fee schedule rate or an amount derived from another payment model. Actual plan reimbursement may be less than this base maximum amount, because of applicable adjustments.
- ***Medicare Physician Fee Schedule (MPFS) Relative Value File:*** A file published by the Centers for Medicare & Medicaid Services that includes, among other information, multiple procedure indicator, relative value units and conversion factors for practitioner services. This file is commonly referred to as the relative value unit (RVU) table. Elements of the file that are referenced in this policy include the following:
  - ***CF:*** The conversion factor is the amount shown in the *MPFS Relative Value File* for the appropriate year.
  - ***PE GPCI:*** The practice expense geographic price index for the area where the service was rendered, shown in the *MPFS Relative Value File* for the appropriate year.
  - ***PE RVU:*** The practice expense relative value unit is the amount shown in the *MPFS Relative Value File* for the appropriate year.
- ***Modifier 26:*** Professional component.
- ***Modifier 51:*** Multiple procedures.
- ***Modifier 59:*** Distinct procedural service.
- ***Modifier TC:*** Technical component.
- ***Modifiers X{EPSU}:*** Modifiers XE, XP, XS and XU, collectively.
  - **Modifier XE**: Separate encounter.
  - **Modifier XP**: Separate practitioner.
  - **Modifier XS**: Separate structure.
  - **Modifier XU**: Unusual non-overlapping service.

- ***Practice Expense (PE):*** One of three groups of inputs used to determine pricing of services covered under the Medicare Physician Fee Schedule (MPFS); PE accounts for costs of running a practice (other than expenses associated with professional liability and the practitioner's actual work), including rent, equipment, supplies and staff costs other than the work performed by the practitioner personally.
- ***Professional Component (PC):*** The professional component of a service is for practitioner work interpreting a diagnostic test or performing a procedure and includes indirect practice and malpractice expenses related to that work.
- ***Same Provider:*** A single qualified practitioner or multiple practitioners within the same group practice.
- ***Technical Component (TC):*** The technical component of a service is for all non-practitioner work and includes administrative, personnel and capital (equipment and facility) costs and related malpractice expenses.

References

- Centers for Medicare & Medicaid Services website. Medicare Claims Processing Manual. Chapter 5 – Part B Outpatient Rehabilitation and CORF/OPT Services. Section 10.7 – Multiple Procedure Payment Reductions for Outpatient Rehabilitation Services. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Medicare Claims Processing Manual. Chapter 12 – Physicians/Nonphysician Practitioners. Sections 20.1 and 40.6. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Medicare Physician Fee Schedule (MPFS) Relative Value Files. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Transmittal 995, Change Request 7442. Multiple Procedure Payment Reduction (MPPR) on Certain Diagnostic Imaging Procedures. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Transmittal 1104, Change Request 7747. Application of the Multiple Procedure Payment Reduction (MPPR) on the Professional Component (PC) and Technical Component (TC) of Certain Diagnostic Imaging Procedures to Physicians in the Same Group Practice. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Transmittal 1149, Change Request 7848. Multiple Procedure Payment Reduction (MPPR) on the Technical Component (TC) of Diagnostic Cardiovascular and Ophthalmology Procedures. www.cms.gov.
- Centers for Medicare & Medicaid Services HCPCS Level II and associated publications and services.
- American Medical Association's CPT® and associated publications and services.

General Humana Resources

- Availity – Providers can register for access to information on a variety of topics such as eligibility, benefits, referrals, authorizations, claims and electronic remittances.
- Claims processing edit notifications – Alerts of upcoming claims payment changes are posted on the first Friday of each month.
- Claims resources – Providers can find information on referrals, authorizations, electronic claim submissions and more.
- Making it easier – This page contains an educational series for providers and healthcare professionals.
- Medical and pharmacy coverage policies – Humana publishes determinations of coverage of medical procedures, devices and medications for the treatment of various conditions. There may be variances in coverage among plans.
- Publications – This page can help you find our quarterly newsletter, provider manual and other resources to help you do business with us.

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